

A 35-year-old, male, presented with left sided pleuritic chest pain, -10 cough and dyspnea. His labs revealed presence of HIV. Chest X-ray showed Left sided cavity surrounded by consolidation, one of common causative :organisms in this case is

- a- Pneumocystis jirovecii (PCP). ☐
- b- Staphylococcus aureus. ☒
- c- Hemophilus influenza. ☐
- d- Streptococcus pneumonia. ☐

محو التحديد

?Shunt occurs when -11

- a- Arterial blood mixes with venous blood (left to right) ☐
- b- Venous blood mixes with arterial blood (right to left) ☒
- c- A combination of both conditions above ☐
- d- Ventilation and perfusion are equal ☐

محو التحديد

:High ventilation/Perfusion mismatch is -12

- a- Ventilation without perfusion ☒
- b- Perfusion without ventilation ☐
- c- Ventilation and perfusion are equal ☐
- d- A cardiac output higher than minute ventilation. ☐

محو التحديد

A 63 year old man has had progressively worsening dyspnea over the -13 past 10 years. He has noticed a 5kg weight loss in the past 2 years. He has a chronic cough with minimal sputum production and no chest pain. On physical examination, he is afebrile and normotensive. A chest radiograph shows extensive interstitial disease. Pulmonary function tests show diminished capacity, and restrictive dysfunction. Increased exposure to ?which of the following pollutants is most likely to produce these findings

a- Carbon monoxide ☐

b- Ozone ☐

c- Asbestosis ☒

d- Tobacco smoke ☐

محو التحديد

Which of these statements regarding the management of non- -14 ?idiopathic pulmonary fibrosis is most accurate

a- Lung transplantation is not considered appropriate patients in severe stages of the disease ☐

b- Antifibrotic agents may reduce lung function decline ☒

c- Pulmonary rehabilitation is not of value. ☐

d- Vaccines should be avoided due to the high risk for side effects and complications ☐

محو التحديد

Which of these subgroups of patients with idiopathic pulmonary fibrosis .15 ?may be considered for long-term oxygen therapy

a- Patients with clinically significant resting hypoxemia ☒

b- Patients who smoke ☐

c- Patients with exertional hypoxemia ☐

d- Patients who do not respond to pulmonary therapy ☐

محو التحديد

?Which one of the following is not true regarding lung tumors .16

- a. The most common benign lung tumors are pulmonary hamartomas, lung neurofibromas, or lipomas ☒
- b. 70% of lung tumors are malignant ☐
- c. Bronchogenic Carcinomas often present late when local invasion and metastases are already present ☐
- d. 5- years survival is less than 15% ☐

محو التحديد

?Which one of the following is not true regarding lung tumors -17

- a. 97% of lung tumors are malignant. ☐
- b. Bronchogenic carcinomas often present late when local invasion and metastases are already present. ☐
- c. Lung cancer is by far the number one cancer killer. ☐
- d. 50% are due to smoking ☒

محو التحديد

**Which of the following is the most accurate about the small pleural .18
effusion not required thoracocentesis**

- a- Recent thoracic or abdominal surgery. ☐
- b- Too small to safely aspirate. ☒
- c- In clinically stable patients. ☐
- d- Presence can be explained by underlying congestive heart failure. ☐

محو التحديد

**What is the most accurate enzyme to rule out TB in case of exudative .19
?lymphocytic pleural effusion**

- a- Amylase. ☐
- b- Adenosine deaminase. ☒
- c- Pepsin. ☐
- d- Lipase. ☐

محو التحديد

**What is the most common cause of simple parapneumonic exudative .20
?pleural effusion**

- a- Viral infection. ☐
- b- Fungal infection. ☐
- c- Bacterial infection. ☒
- d- Tb infection ☐

محو التحديد

:In which situation surgical management of bronchiectasis is preferred - 1

- a. Localized bronchiectasis ☒
- b. Cystic fibrosis ☐
- c. Allergic bronchopulmonary aspergillosis ☐
- d. Bilateral bronchiectasis ☐

محو التحديد

Upper lobe predominate bronchiectasis in -2

- a. allergic bronchopulmonary aspergillosis ☐
- b. foreign body inhalation ☐
- c. William Campbell syndrome ☐
- d. Cystic fibrosis ☒

محو التحديد

?Which of the following is NOT a characteristic of asthma -3

- a. Increase in IgG immunoglobulins. ☒
- b. Airway hyperresponsiveness. ☐
- c. Infiltration of eosinophils into the airways. ☐
- d. Increased mucus production. ☐

محو التحديد

A 30-year-old patient with a history of mild persistent asthma (baseline-4 peak expiratory flow rate of 85%) presents to the emergency department with shortness of breath and wheezing that has not relieved by her albuterol inhaler for the past 12 hours. She was able to tolerate pulmonary function tests and a set was performed. Which of the following is the most likely test result?

- a. Decreased FEV1, normal/increased FVC, decreased FEV1: FVC ratio, with post bronchodilator. ☒
- b. FEV1 increased by 13%. ☐
- c. Decreased residual volume and total lung capacity. ☐
- d. Increased FEV1 increased FVC, normal FEV1: FVC ratio. ☐

محو التحديد

Which one of the following is risk for primary pneumothorax -5

- a. Chest trauma ☐
- b. Sub plural bleb or bullae ☒
- c. COPD ☐
- d. Asthma ☐

محو التحديد

In case of large pneumothorax with minimal symptom the treatment of -6 :choice is

- a. Refer to cardiothoracic surgery ☐
- b. Oxygen therapy ☒
- c. No intervention just observation ☐
- d. Insert chest tube ☐

محو التحديد

?Which one of the following is a first line drug for TB treatment -7

- a. Pyrazinamide ☒
- b. PASA ☐
- c. Kanamycin ☐
- d. Cycloserine ☐

محو التحديد

:GHON focus is generally located in -8

- a. Upper part of the lower lobe ☐
- b. Lower part of the upper lobe. ☐
- c. Upper part of upper lobe ☐
- d. A and B ☒

محو التحديد

A 60-year-old, male, Shisha smoker, presented with cough and toxic -9 symptoms. Chest X-ray showed upper right sided cavity with air-fluid level, sputum microbiological culture revealed no growth. Patient started broad spectrum antibiotic with no improvement. One of possible causative ?organisms is

- a- Staphylococcus aureus. ☐
- b- Hemophilus influenza. ☐
- c- Streptococcus pneumonia. ☐
- d- Mycobacterium tuberculosis bacilli. ☒

محو التحديد